



Fiona Stanley Fremantle Hospitals Group

Guideline

Mother Baby Unit admissions

Reference #: FSFH-HW-GUI-0020

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Mother Baby Unit	Medical, Nursing, Allied Health

1. Introduction

The Mother and Baby Unit (MBU) is a state-wide service providing eight authorised beds for the inpatient treatment of patients with acute psychiatric conditions in the post natal period. This document clarifies the MBU admission criteria and referral processes to enable referrers and after hours clinicians to assess patient suitability for admission to the Unit.

2. Terminology

Postnatal	The period immediately following the birth of an infant to 12 months after the birth.
Post partum psychosis	A severe low prevalence psychotic disorder generally occurring in the first few weeks after the birth of a baby
Severe mental illness	Mental disorders causing significant acute or chronic impairment including but not limited to, schizophrenia, psychosis, bipolar mood disorder, schizoaffective disorder and postpartum psychosis.

3. Guideline

3.1. General information

A safe ward environment must be maintained on the MBU due to the vulnerability of young infants on the ward.

- Patients are admitted to the MBU for the assessment and treatment of mental health disorders affecting mothers who are the primary caregivers for their infants in the first year postnatally.
- The service is primarily for patients living within the South Metropolitan Health Service (SMHS) area and rural/remote patients.
- The King Edward Memorial Hospital (KEMH) MBU services the North Metropolitan Health Service and rural/remote patients.

3.2. Admission criteria

- Includes: Mothers of babies aged 12 months or younger who have a moderate to severe mental health issue that impacts on their level of functioning and/or their ability to care for their baby.
- Mothers who are the primary caregiver of their baby.
- Mother and baby are medically cleared from any infectious disease or illness.
- Women and their baby/babies will be admitted to the MBU together unless the baby is admitted to the FSH Neonatal Unit. In this case the mother may be admitted prior to the baby's discharge from the Neonatal Unit.
- Antenatal patients who are greater than 32 weeks gestation will be considered for MBU admission if clinically indicated and delivery is planned at FSH.

Refer to [Appendix 1: MBU Admission Criteria](#)

3.3. Exclusion Criteria

- Cases where a parenting assessment is the primary purpose of the admission.
- Mothers:
 - with a primary substance use disorder.
 - who have a history of violent, aggressive or severely disruptive behaviour.
 - displaying behaviour that may be dangerous to their baby or who pose a risk to the safety of the ward environment.
 - who are currently not the primary caregiver for their infant as Child Protection and Family Services (CPFS) have assessed there to be current care and protection concerns
- Baby is 12 months or older or walking, whichever comes first.
- Mothers and babies that are medically/physically unwell and are requiring hospitalisation for physical illness or are an infection risk.

Where the patient meets exclusion criteria but the referrer assessment has deemed the patient likely to benefit from MBU admission, the MBU Consultant Psychiatrist will review the referral information and liaise directly with the referrer to determine if MBU is clinically indicated.

3.4. Referral Process

- 3.4.1. The MBU Clinical Nurse Specialist (CNS) will assess a patient's eligibility for referral (refer to [Appendix 2](#)) and inform the referrer of the referral process (refer to [Appendix 3](#)).
- 3.4.2. The referrer will be informed that the MBU is a tertiary centre and does not have an acute response service. Referrers will be directed to the Mental Health Emergency Response Line (MHERL) or the nearest Emergency Department (ED) if the patient requires crisis intervention.
- 3.4.3. The MBU referral form can be accessed via the Fiona Stanley Hospital [Mother and Baby Mental Health Unit referrals](#) webpage or a copy can be sent to the referrer on request.
- 3.4.4. The referral form must include a current risk assessment and a mental health assessment including presenting issues and a mental state examination. Once completed the form is faxed or emailed to fsh.mhmotherbabyunit@health.wa.gov.au
- 3.4.5. Referrals are triaged by the MBU Multidisciplinary team daily from Monday - Friday.
- 3.4.6. The MBU CNS will:
 - contact the referrer by the next business day and clearly communicate the status of the referral i.e. accepted, not accepted or to request more information
 - advise the referrer that governance of the patient remains the responsibility of the referrer until the patient is admitted to the MBU.
- 3.4.7. The MBU will be responsible for adding the patient to the MBU waitlist or suggesting alternative mental health assessment and treatment services.
- 3.4.8. Referrals will be prioritised according to clinical presentation and level of risk.
- 3.4.9. The MBU will collaborate with KEMH MBU to ensure that admissions of patients with urgent clinical need are prioritised.

3.5. Urgent referrals for after hours admission

- 3.5.1. Urgent referrals after hours will be redirected to the Hospital Out of Hours Team (HOOT) Manager. Refer to [Appendix 4](#).
- 3.5.2. After hours presentations to the ED that may require admission to the MBU will be assessed by the on-call Psychiatric Registrar, taking into consideration suitability for admission and whether the patient meets the admission criteria.

3.5.3. All patients referred must be medically cleared prior to MBU admission.

3.5.4. All after-hours MBU admissions must be authorised by the on call Consultant Psychiatrist.

3.5.5. Acute Psychiatric presentations assessed as requiring separation of the mother from her baby due to risk to the baby, should be considered for admission to an Adult Mental Health Unit prior to referral to the MBU.

4.Compliance/Performance Monitoring

Compliance will be monitored by the MBU CNS through routine clinical incident reviews and patient feedback.

5.Related Policy Documents

FSFHG:

- Admission and Bed Allocation – FSFHG Bed Management (FSFHG GP 01)
- Clinical handover (FSFH-HW-POL-0035)
- Antenatal Clinic: Referrals to Social Work, Clinical Psychology and/or Mental Health (FSH-MENH-GUI-0004)

6.Related Standards

NSQHS Standards:

- Clinical Governance
- Comprehensive Care
- Recognising and Responding to Acute Deterioration

National Standards for Mental Health Services

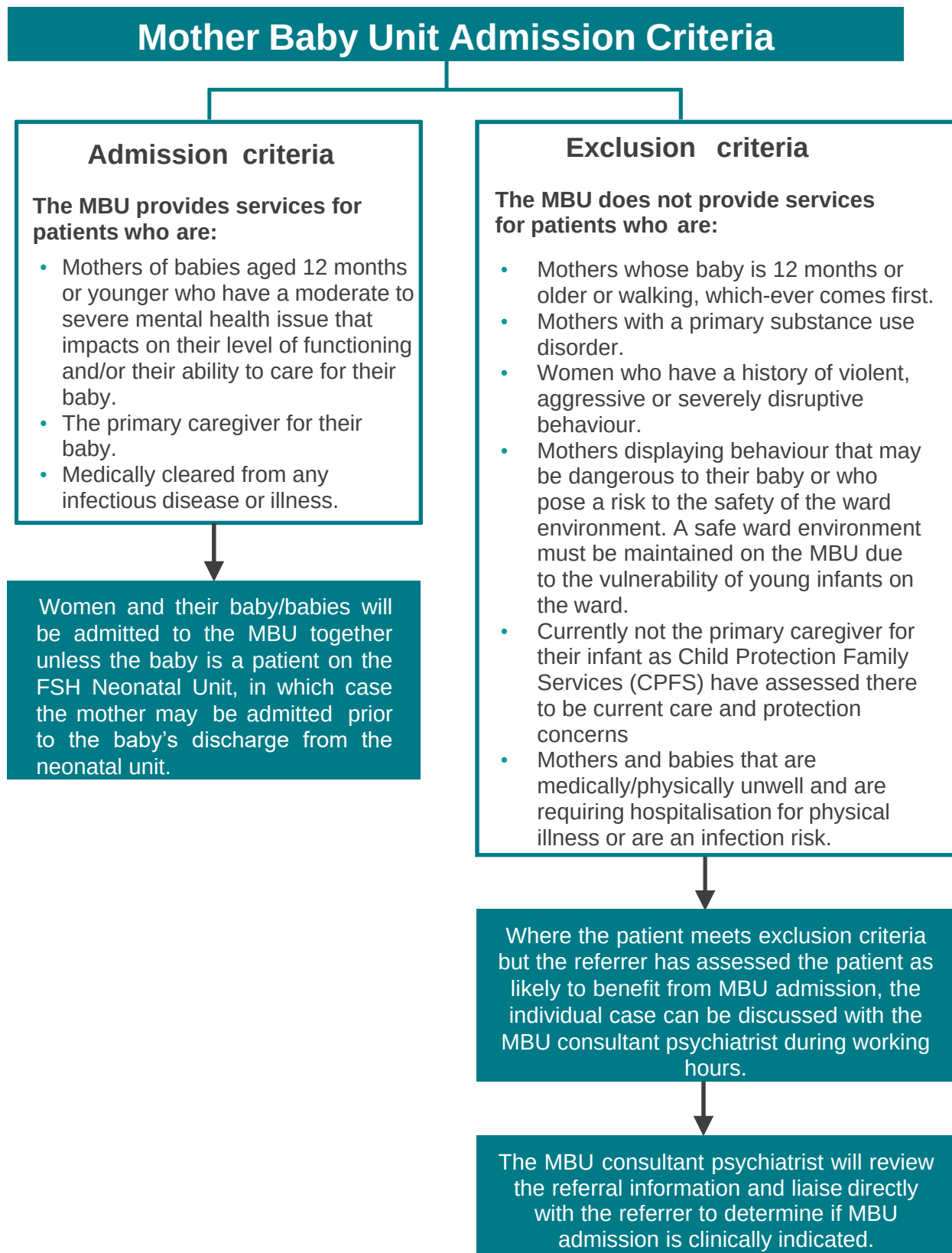
Chief Psychiatrists Standards for Clinical Care

7.Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	08/2019	CNS, Perinatal	Nursing and Midwifery Practice Governance Committee	FSFHG Policy Committee	08/2022
2	08/2022	CNS, Perinatal		Service 5 Safety Quality and Risk Committee	09/2025
3	11/2022	Policy Team	FSFHG HEC		09/2026

8. Appendices

Appendix 1: MBU Admission Criteria



Appendix 2: Eligibility for Referral

Mother Baby Unit (MBU) Referral Guide

Eligibility for Referral to MBU – Referrers guide

Step 1: Eligibility

Mother, who is the primary caregiver for her infant (<12months), presenting with a mental health condition requiring inpatient treatment/cannot be adequately or safely managed in the community

Step 2: MBU exclusion criteria

Exclude patients who have:

- A current infection risk **and/or** physical illness requiring treatment/hospitalisation
Action: refer once infection has resolved **and/or** following stabilisation of physical illness.
- Severely disruptive behaviour or aggression due to the risk to their baby or other infants
Action: Refer to adult mental health unit with alternative care arranged for the baby. Refer to MBU again once risk of aggression has resolved.
- A primary substance disorder

Child Protection and Family Services (CPFS) are currently involved - **Yes/No**

- If yes, provide details of CPFS involvement including CPFS case worker contact details and any current care and protection concerns with the referral

Step 3: Informed consent

Discuss the need for a mental health admission to the MBU with the mother/family, provide MBU Patient Information pamphlet, and obtain consent for referral.

Step 4: Communication

Inform mother/family that the mother and baby will be admitted together.

If the mother is being referred as an involuntary patient to the MBU and does not have capacity to consent, obtain consent for MBU admission of baby with mother from the infant's parent/legal guardian.

Step 5 Refer Patient

Appendix 3: Referral Process

Mother Baby Unit (MBU) Referral Process

Step 1

Contact the MBU Clinical Nurse Specialist on **(08) 61521583** to discuss the referral

Step 2

Complete the MBU referral which can be accessed via FSH MBU webpage or on request to the MBU Clinical Nurse Specialist.

The referral form includes:

- » **A current risk assessment**
- » **A current mental health assessment, this includes presenting issues and a mental state examination.**
- » **Summary of treatment to date**

Step 3

Once completed the form is faxed to **(08) 61524867** or emailed to fsfsh.mhmotherbabyunit@health.wa.gov.au

Step 4

All referral are triaged by MBU Multidisciplinary team daily Monday – Friday.

Step 5

The MBU CNS will notify the referrer of the outcome of the referral (i.e. accepted or not accepted) by the next business day.

Accepted

If the referral is accepted, the patient will be offered an admission or added to the bed waitlist. The referrer is advised governance remains with them until admission

Not Accepted

If the referral does not meet inclusion criteria, alternative mental health assessment and treatment services will be suggested.

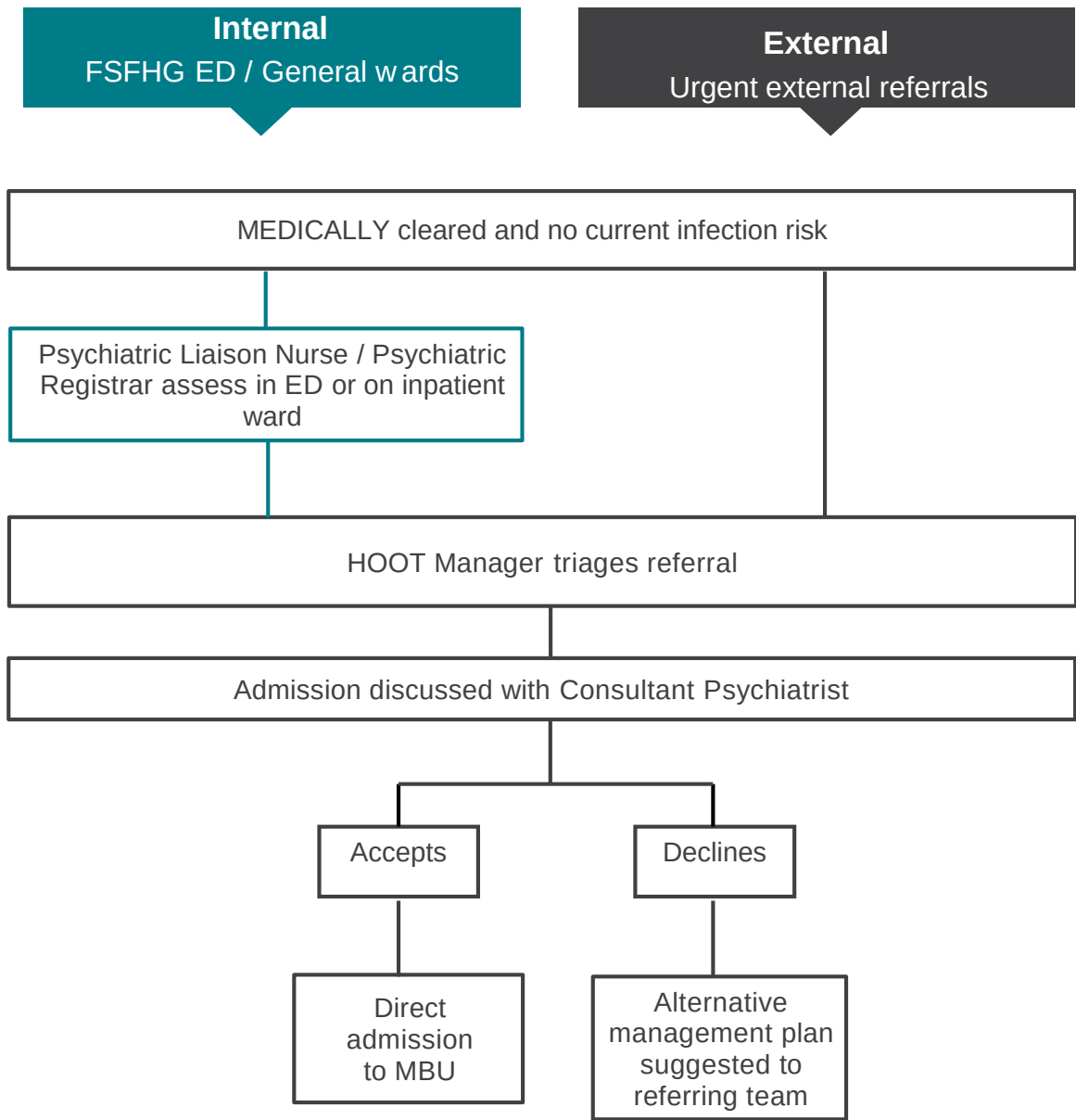
Step 6

When a bed is available, the MBU CNS will contact the woman directly and will notify the referrer of the admission.

If there are no available MBU beds and the MBU is unable to admit an urgent referral, FSH MBU CNS will liaise with the KEMH MBU to determine if they have capacity to take the admission.

Appendix 4: After Hours Admission Procedure

Mother Baby Unit (MBU) Afterhours Admission



NB. Patients presenting with acute mental health issues and high levels of behavioural disturbance, who are assessed as requiring separation of the mother from her baby due to risk to the baby should be considered for an adult mental health admission prior to referral to the MBU.